

SHARE OF HEALTH AND FAMILY WELFARE IN TOTAL EXPENDITURE OF STATES IN FY25

• Share in revenue expenditure (₹Cr)	2,62,711
• Share in revenue expenditure (%)	6.15
• Share in capital expenditure, including loans and advances (₹Cr)	40,068
• Share in capital expenditure (%)	4.72
• Total share (₹Cr)	3,02,779
• Share in total expenditure (%)	5.91

SOURCE CAG PUBLICATION ON STATE FINANCES 2024-25

OUT-OF-POCKET EXPENDITURE ON HEALTH REMAINS HIGH

• Expenditure per hospitalisation case (excluding childbirth) during the last 365-day period

INSTITUTION CATEGORY	AVERAGE OUT-OF-POCKET MEDICAL EXPENDITURE (₹)		
	Rural	Urban	Rural+Urban
Govt. hospital/ Public health facilities	6,905	6,032	6,631
Charitable/ Trust/ NGO-run hospital	40,203	38,419	39,530
Private hospital (incl govt empanelled)	47,710	54,999	50,508
All	31,484	38,688	34,064

SOURCE NSS REPORT NO. 596: HOUSEHOLD SOCIAL CONSUMPTION: HEALTH

An analysis by PRS Legislative Research of the Demand for Grants for health and family welfare for FY27 points out that schemes like the National Health Mission, PM-ABHIM, NHM (for primary health infrastructure), and PMSSY (setting up new AIIMS) saw under-utilisation of funds in FY26.

Experts believe that apart from higher and better



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“Government health expenditure has increased from 28.6% of the total health expenditure in 2013-14 to 43.7% in 2022-23, while per-capita government spending has nearly tripled over the same period”

allocation of funds, we need more targeted initiatives and, more importantly, strengthen healthcare closer to where people live.

Kalmath of Grant Thornton Bharat says there is a need for investment in primary care, health and wellness centres, frontline health workers, diagnostics and essential medicines. “This is consistent with current spending patterns, with around 51% of government health expenditure directed towards primary care,” he says, adding that technology is also increasingly being used as an enabler of access and service delivery. Telemedicine, digital health records, and platforms such as the Ayushman Bharat Digital Mission can bridge the gaps in healthcare delivery.

Another important consideration is the role of states, which account for nearly two-thirds of government health expenditure and remain the primary drivers of healthcare delivery, he says.

With India’s population seen to age over the coming decades and the elderly population seen to double to over 300 million by 2050, public healthcare initiatives would also have to deal with this demographic shift.

But the question remains. Can India not just become richer but also healthier before it turns older? That hope remains. **BT**

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